

Drug Interaction Reference - Internal Memo

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Section 1: Background

This document summarizes interaction profiles for selected anticoagulant and antiplatelet therapies in patients on concurrent SSRI medications.

Section 2: Warfarin + SSRI

Warfarin combined with most SSRIs (fluoxetine, sertraline, paroxetine) carries a **CLINICALLY SIGNIFICANT** increased bleeding risk due to platelet dysfunction. Standard practice is to monitor INR more frequently (every 3-5 days for the first 2 weeks) and consider dose adjustment.

Section 3: Aspirin + Citalopram

Low-dose aspirin (81mg/day) with citalopram has a moderate interaction profile. Both agents independently affect bleeding risk; concurrent use should be evaluated case-by-case, typically with a PPI co-prescription if the patient has GI bleeding history.

Section 4: Clopidogrel + Fluoxetine

Fluoxetine is a CYP2C19 inhibitor and may **REDUCE** the active metabolite of clopidogrel, potentially reducing antiplatelet effect. Alternative

combinations (sertraline, escitalopram) are preferred when feasible.

Warfarin + sertraline

elevated

Action

increase INR monitoring

Warfarin + fluoxetine

elevated

increase INR monitoring

Aspirin + citalopram

moderate

evaluate; consider PPI

Clopidogrel + fluoxetine

reduced efficacy

prefer sertraline/escitalopram

Section 5: Summary Table